

Philipp Hilbert. *Platform PM who speaks both FHIR and Kubernetes.*

Positioning, ten years building data platforms in regulated environments – now bringing that depth to healthcare interoperability at the infrastructure layer.

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To the 1upHealth Product team,

Healthcare data is one of the few domains where poor data quality directly harms people rather than just degrading conversion rates. That framing matters to me, and it is part of why the PM role at 1upHealth is compelling. You are building the infrastructure layer that lets clinical and claims data move correctly across an ecosystem that has historically treated interoperability as an afterthought. That is a hard, important problem, and it requires a PM who can hold the technical detail and the customer story at the same time.

My most directly transferable experience is at Rohde & Schwarz, where I owned the AI and data management platform for the FCAS defence programme. The core challenge there was the same one 1upHealth faces: ingesting multimodal data from heterogeneous sources, normalising it into a consistent structure, and making it auditable enough that downstream ML workflows could trust the provenance of every record. I designed the transformation pipelines, wrote the data provenance and audit-trail requirements, and introduced GitOps-based rollout that cut deployment times from several days to under forty minutes. I did that as effectively the sole PM on a cross-functional team of architects, data scientists, and backend engineers – which meant constant translation between what compliance and stakeholders needed and what engineering could feasibly build in a given sprint.

At the Federal Employment Agency I added ETL pipeline ownership in a more analytics-heavy context: introducing Dagster for automated indicator generation, migrating the Arbeitsmarktmonitor to Kubernetes, and serving as Deputy IT Security Officer. The combination of data-lineage accountability and delivery ownership is directly relevant to a platform that processes sensitive clinical records for payers and providers operating under strict regulatory obligations.

Earlier, at Emil Group, I took on a struggling InsurTech SaaS platform mid-delivery. I rebuilt sprint discipline, split an unwieldy seventeen-person team into effective Scrum units, re-established velocity tracking, and ran weekly product workshops to rebuild client trust. The

Claims Center module I designed from concept to production rollout in that environment is the kind of end-to-end ownership the 1upHealth PM role describes. I mention it because the ability to stabilise delivery while simultaneously advancing the product roadmap is a rare combination, and it is one I have demonstrated under pressure.

I am honest about where my gap sits: I have not yet shipped a product with FHIR or HL7v2 as the primary integration layer. What I bring instead is a track record of learning regulated technical standards quickly, designing around their constraints, and making them legible to both clinical and engineering audiences. Given that 1upHealth explicitly values startup pragmatism and the ability to translate complex technical topics for mixed audiences, I believe the transferable mechanics are strong.

I would be glad to walk through any of these projects in more detail, or to discuss how I would approach the first roadmap conversation with your engineering and customer success teams.

Best regards, *Philipp Hilbert*
